Naloxone – Intramuscular

– see separate IV MONOGRAPH if intravenous administration is required.

FORM

Ampoule containing 400 micrograms/ml injection

INDICATION

At resuscitation - reversal of respiratory and CNS depression in neonate following maternal opioid use in labour.

DOSE RANGE

AGE	DOSE	FREQUENCY	ROUTE
Neonate	200 microgram *	Single dose to be given at birth	I.M. into the anterior thigh

*Note: for very premature infants or infants with a low birth weight, a dose of 60 micrograms/kg can be used

RECONSTITUTION

Already in solution (400micrograms/ml)

ADMINISTRATION

200micrograms = 0.5ml of pre-made solution. IM into anterior thigh

FURTHER INFORMATION

Caution - the half-life of a narcotic drug may be longer than that of Naloxone.

In apnoeic neonates an adequate airway should be established before treatment.

Onset of action when given IM is approximately 3-4 minutes and produces a more prolonged effect than IV administration. The onset is faster if given IV but duration of action is shorter.

LICENSED STATUS

Licensed.

APPLICABLE POLICIES

West of Scotland Neonatal Guidelines

Consult local policy if applicable

Document Number:	002	Supersedes:	001
Prepared/Checked by:	Olivia Scales/Susan Kafka/Natalie Donald	Approved by	WoS Neonatal Pharmacist Group
Date prepared	September 2026	Review date	September 2029

Administer reconstituted solutions immediately.
All vials, ampoules and infusion bags are for single use only unless otherwise stated.

Dose may vary depending on indication, age, renal function, hepatic function, and concomitant medications.
This monograph should be used in conjunction with the terms of reference document prepared by the West of Scotland Pharmacist Group. Information is correct at the time of publication and as per local practice agreement.
For further advice please contact your clinical pharmacist or pharmacy department